

The Parkinson's Caregiver's Handbook - Printable Charts and Cards

From *The Parkinson's Caregiver's Handbook* by Anne Merriweather (Oro Enigma Publishing). Print as many copies as you need.

THE NEVER-GIVE CARD

Cut this out and put it in your wallet. This is the page that matters most.

This person has PARKINSON'S DISEASE.
1. His medication is TIME-CRITICAL. Give at HIS times, not the ward round times. A dose 40 minutes late can leave him unable to move or swallow.
2. Please DO NOT give dopamine-blocking drugs. Ask before anything for sickness or agitation.
3. Please DO NOT stop his Parkinson's medication. Abrupt withdrawal can cause a dangerous reaction. If he is nil by mouth or vomiting, ask about a rotigotine patch urgently.

Drugs to question

Category	Examples
Anti-sickness that blocks dopamine	metoclopramide, prochlorperazine, older phenothiazines
Most antipsychotics	haloperidol, chlorpromazine, risperidone, olanzapine

Generally used instead - nausea: ondansetron (domperidone where available). Psychosis: quetiapine, clozapine, or pimavanserin (US).

Confirm this list with your own specialist nurse or pharmacist and write their version here:

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Medication Timing Chart

Write times as **times**, not "three times a day". Update the day anything changes.

Name (brand and generic)	Strength	How many	Exact time	Notes (before food? with a biscuit?)

Date this chart was updated	
Copies are kept: fridge / bag / hospital kit	

Hospital Admission Sheet

Hand this to the nurse on arrival, and again at every shift change.

Name	
Date of birth	
Diagnosis	Parkinson's disease, diagnosed
Neurologist / hospital	
Parkinson's nurse specialist + number	
Next of kin + number	
Allergies	

Parkinson's medication - TIME CRITICAL

Medicine	Dose	Exact times

Other medication

Medicine	Dose	Times
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Please note

This medication is time-critical. Please allow self-administration if hospital policy permits.
Please check anything for sickness or agitation against the never-give list before giving it.
Please do not stop the Parkinson's medication. If nil by mouth, please ask about a rotigotine patch.

On a good day he can: (walk unaided? dress himself? speak normally? This tells staff what his baseline is.)

The Dose Diary

Fill this in for **three or four days before an appointment**. It changes what the specialist can do more than any conversation.

Mark each hour: **ON** (moving well) · **OFF** (slow, stiff, stuck) · **DYS** (involuntary movements). Mark **T** when a tablet is taken and **M** for a meal.

Time	Day 1	Day 2	Day 3	Day 4
06:00				
07:00				
08:00				
09:00				
10:00				
11:00				
12:00				
13:00				
14:00				
15:00				
16:00				

17:00				
18:00				
19:00				
20:00				
21:00				
22:00				

What to look for when you read it back: a dose that reliably fails (look at what was eaten near it), a bad patch always at the same point before the next tablet, dyskinesia always at the same point after one.

Freezing Cue Card

For family, carers, and anyone who looks after him. Put a copy where helpers will see it.

DO NOT pull him forward by the arm. His feet are stuck and his body is not - pulling is how people fall.

Instead:

1	Stop. Let him stand still. Trying harder makes freezing worse.
2	Rock gently side to side, weight from one foot to the other.
3	Count aloud - "one, two, one, two" - or march.
4	Give him something to step over - your foot, a line, a stick laid down, a laser pointer.
5	Use a rhythm - music with a strong beat, or a metronome.
6	Aim for a big first step , not a small one.
7	Remove the hurry. "There's no rush. I'll wait."

Which cue works best for him:

Freezing is worse: starting off · turning · doorways and thresholds · approaching a chair or toilet · doing two things at once · when rushed · **when the medication is low**

Falls Checklist

Room by room

	Done
Loose rugs removed (not just flattened)	
Clear route to the bathroom and the bed	
Night lighting on the route to the toilet	
Nothing at ankle height in walkways	
Furniture he leans on is steady - and the thing he actually grabs is secure	
Grab rails: bathroom, toilet, stairs	
Shoes fit, firm soles, no flapping slippers	
Urinal or commode by the bed if he gets up at night	

Habits

Turn in a wide arc, like a car - never pivot on the spot	

No carrying, talking or hurrying while walking	
Stand up in stages: sit on the edge, wait, then rise holding something	

After a fall, record this

Date / time	Where	What he was doing	How long since the last tablet	Injury?

If falls cluster at the same time of day, that is a medication conversation, not just a hazard one.

Questions for the Next Appointment

Take the dose diary and three specific, dated observations.

What has changed since last time

What	When it started	How often

Questions worth asking

Is this symptom the Parkinson's, the medication, or something else?
Is the timing of the doses still right, given the diary?
Should we be referred to physiotherapy / speech therapy / occupational therapy / a dietitian yet?
Is he a candidate for any advanced therapy (DBS, apomorphine, intestinal gel)? If not, why not?
What should make us ring you rather than wait for the next appointment?

Say the embarrassing things. Hallucinations, gambling, spending, sexual behaviour, low mood. They are specific, common and treatable, and families staying silent out of loyalty is the main reason they go unmanaged for years.